

PROJECT 100 ENCYCLOPEDIA

The Complete Full Encyclopedia - V2.0

Full Detailed Version + Complete Clinical Analysis merged into one definitive volume.

Nutrition - Training - Pills - Supplements - Vectors - Laboratory integrity.



VERSION 2.0 - 3 OCTOBER 2026 - SUPERSEDES PLATINUM OMNIBUS V18700 PUBLISHING EDITIONS AS THE READING MASTER

Prime directive

100 lb of bioimpedance-estimated skeletal muscle mass with body fat below 15 percent.
Personal targets tracked under standardized conditions - not medical thresholds or deadlines.

Operational core - verified

Zinc Supreme removed. Separate copper paused. Boron locked at 1 x 3 mg. Known totals always shown with limits. Publication validates records and arithmetic; it does not grant clinical clearance.

How V2.0 was built: every operational dose from V18700 (30 September 2026) is retained. The clinical layer previously held only in the separate 33-page Protocol Analysis - mechanism, interaction, toxicological margin and laboratory consequence - is now woven into every chapter. The rest of the V2.0 set (Guidebook, Simplified Structure, Marketing Slider, Website documents) is generated from this Encyclopedia.

How to read this Encyclopedia

Every chapter carries three layers. Read them in order and you will never mistake a mechanism for a prescription.

Layer 1 - Operational rule

What is actually done, on which day, at what recorded dose. Example: boron is one 3 mg capsule. Training day and rest day are different operating systems.

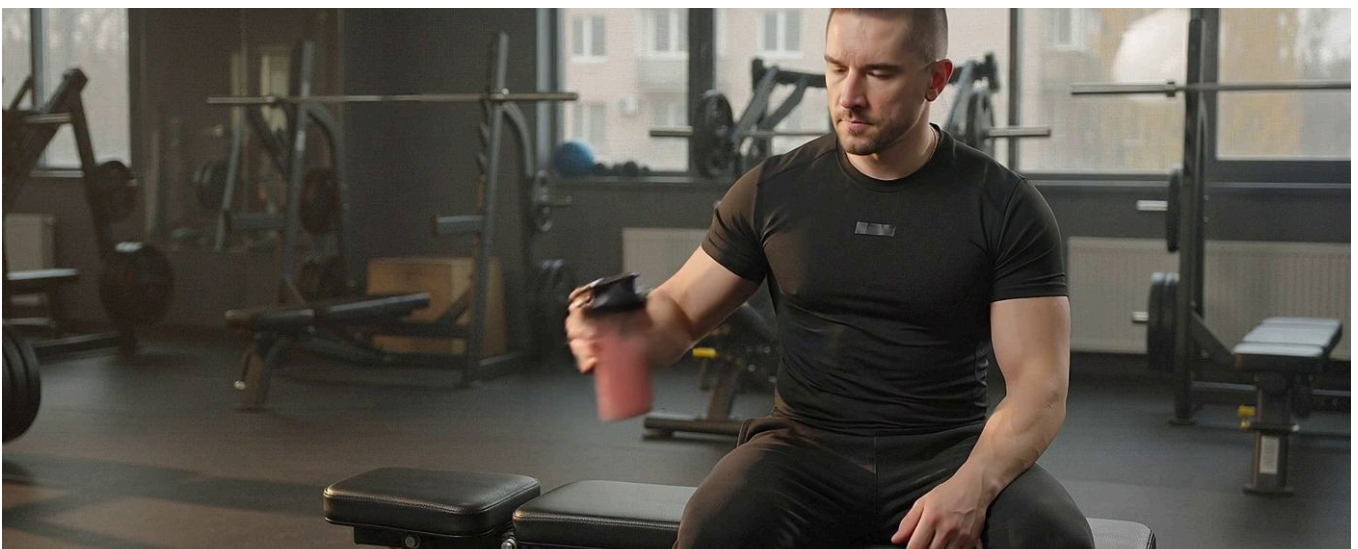
Layer 2 - Dose record

Label basis: capsule count versus serving size, market version (Canadian label where noted), and whether use is daily, intermittent or optional. Half-dose Medi-C and rare Hey Relax remain optional.

Layer 3 - Clinical analysis (new in V2.0): why the rule exists, what the compound does in the body, what it interacts with, where the toxicological margin sits, and what it does to a laboratory test - cholinergic load, CYP3A4 and P-glycoprotein inhibition by piperine, the alpha-lipoic acid to insulin autoimmune syndrome pathway, the magnesium plus vitamin C osmotic chain, and the biotin assay interference pattern.

The non-negotiable reading rules

- Known totals are partial label calculations, not a measured whole diet. Food review remains deferred.
- Symptoms override the schedule: worsening weakness, jaundice, significant pain, recurrent watery stools or disrupted sleep require assessment, not dose escalation.
- March laboratory results predate the May supplement stack. Do not read them as a verdict on the current stack.
- Tell the laboratory about biotin (425 mcg total daily) and creatine HCl (1.5 g daily) before testing. There is no universal 72-hour biotin hold - follow assay-specific instructions.
- A removed item cannot reappear because an old page includes it. Correct transcription or serving errors immediately.



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Version control

Edition	Date	Status
Platinum Omnibus V18700	30 September 2026	Source of truth for doses. 76 pages.
Protocol Analysis (clinical audit)	September 2026	33 pages. Mechanism layer - now integrated.
Full Detailed Version V2.0	3 October 2026	33 pages. Now merged into this volume.
Complete Clinical Analysis V2.0	3 October 2026	21 pages. Now merged into this volume.
Encyclopedia V2.0 (this book)	3 October 2026	The single master. All derivatives are generated from it.

File naming: PROJECT_100_ENCYCLOPEDIA_V2.0_2026-10-03. Author: Bogdan Dospi, age 46. This documents one person's routine. It is not a prescription.

Introduction - The architecture of return

PROJECT 100 records a personal return to training and a long-term ambition: 100 lb of skeletal muscle mass with body fat below 15 percent. Those numbers express the author's goal. They do not establish a medical threshold, a guaranteed outcome or a deadline.

A difficult personal period disrupted routines. In 2026, attention returned to health and gym training. The current source profile reports age 46 and body weight

90.76 kg. The February publications recorded an earlier baseline of age 45, weight 197.1 lb, skeletal muscle mass 88.2 lb and body fat 22.9 percent. These are separate dated records. This edition does not manufacture a single exact chronology from conflicting earlier dates.

The author reports improvements in recovery, digestion and injury symptoms since rebuilding the routine. Those experiences belong in the journey. They cannot isolate the contribution of each supplement or prove that a large combined stack is harmless. The edition therefore gives every item a recorded dose, use status and evidence limit. Brand changes and label versions matter as much as product names.

The current gym branch uses morning workouts. Vectors V3, V1, V2, V4 and V10 describe that branch when consumed. The rest branch uses V3, V5 and V10 when consumed, with a smaller pill routine. The evening PUMP alternative is inactive. Zinc Supreme is removed, separate copper is paused, and boron is one 3 mg capsule. Optional half-dose Medi-C and rare Hey Relax use remain optional. This publication does not turn them into daily requirements.

Clinical analysis - rebuilding after a prolonged interruption.

A prolonged interruption can affect strength, nutrition, sleep and recovery. Retraining builds capacity through repeatable mechanical loading, adequate energy and sleep. The 2026 ACSM position supports individualized

resistance training and regular participation; it does not validate any fixed excavation duration or load percentage. Progress is judged against the author's own comparable measurements, function and health.



Chapter 1 - The prime directive - what 100 means

The target is 100 lb of reported skeletal muscle mass, not lean body mass or total body weight.

Bioimpedance analysis (BIA) estimates skeletal muscle mass (SMM) from an algorithm sensitive to intracellular and extracellular water, glycogen and electrolyte shifts. It does not directly weigh muscle tissue. Under multi-gram osmotic supplementation, fluid compartment changes can move the estimate without any change in contractile protein. Use the same device and repeat comparable measurements - waking, post-void, pre-intake, similar hydration and meal state - before interpreting a change as tissue growth. Track body weight, waist, performance and symptoms alongside the estimate.

Body fat below 15 percent remains a personal aspiration, not a universal requirement for insulin sensitivity, hormone health or successful training. The historical book used competitive bodybuilding as a visual reference. A competition division cannot establish an individual's biological ceiling. The IFBB distinguishes Men's 212 Bodybuilding from Classic Physique; height and weight tables from Classic Physique must not be transferred to the 212 division. Retain the reference as inspiration only.

Analysis 1A - BIA physics and the 212 reference. Multifrequency BIA models total body water, then derives fat-free mass and SMM by regression. Creatine HCl (1.5 g daily), high-dose electrolytes and carbohydrate loading all expand intracellular water and will inflate SMM readings. This is why the chronometer standardizes measurement timing and why quarterly review beats weekly reaction. The prime directive phrase "Train Like Two. Eat for Two" is retained as historical branding describing ambition and sustained effort. It does not prescribe twice the workload or twice the calories.



Chapter 2 - Excavation and the return to training

Re-establish a sustainable routine after interruption before chasing peak loads.

The excavation phase finds loads, meal patterns and recovery habits that can be repeated. It cannot be measured by a claim that calories now partition perfectly into muscle. The old 4-6 week duration and 65-75 percent 1RM range were planning examples, not demonstrated personal requirements. Current readiness reflects symptoms, prior training, available time and recovery.

Reset the chronometer. A consistent sleep opportunity and morning training window make the day easier to run. Most adults should regularly obtain at least seven hours of sleep, but duration alone does not establish adequate quality. Record awakenings and daytime function. Morning caffeine may still affect sleep in a sensitive person. The IGNITER portion supplies approximately 373 mg caffeine before tea or other sources. A late rescue dose is not a remedy for insufficient sleep.

Digestive recalibration. The liver continues producing bile after cholecystectomy. Some people experience persistent diarrhea or difficulty tolerating large fatty meals, but gallbladder removal alone does not prove pancreatic enzyme insufficiency. The author reports clear digestive benefit from meal enzymes and intermittent probiotic use. Preserve that report without claiming an over-the-counter blend replaces bile or guarantees absorption. Beverage temperature follows comfort; the old mandatory hot-solvent rule is retired.

Analysis 2A - physiology of return. Early strength gains after retraining are dominated by neural recruitment and skill, not new myofibrils. Controlled repetitions, repeatable setups and manageable volume let the operator evaluate tolerance. A slower lowering phase or lighter supported exercise may help evaluation, but neither proves a nerve has healed or a spinal segment is protected. Spacing of heavy axial sessions (historically 48-72 hours) requires individual adjustment when back, elbow or shoulder symptoms are present.



Chapter 3 - Hardware locks and clinical constraints

Symptoms and clinical constraints govern exercise selection before ambition does.

A hardware lock is a symptom or clinical constraint that changes what may be loaded: worsening hand weakness, spinal symptoms, shoulder or elbow pain, and post-cholecystectomy digestion. Locks are recorded, not argued with. The training programme is built around them with supported rows, presses and controlled ranges, and load is progressed only when symptoms remain stable.

Gallbladder removal does not mean bile production stops. The liver produces bile continuously; without a gallbladder there is no storage and concentration step, so bile trickles into the intestine and large fatty meals are harder to emulsify at once. Smaller fat portions, slower eating and meal enzymes are individual experiments. Recurrent watery stools, weight loss, significant pain, jaundice or oily stools deserve assessment.

Analysis 3A - red-flag boundaries. Progressive weakness, bowel or bladder change, saddle numbness, unexplained weight loss, jaundice, fever with abdominal pain, or chest symptoms are not training variables. They stop the session and trigger clinical assessment. Calcium does not neutralize excess zinc, and a comforting hot drink does not establish improved fat emulsification - retired myths stay retired.



Chapter 4 - The chronometer and daily sequence

Two operating systems: morning training branch and rest branch. Clock times are reference anchors, not rigid deadlines.

Anchor	Morning Training Branch	Rest Day Branch	Purpose / constraint
03:30-03:45 Wake Pack	TESTOFX (3 caps), Boron (3 mg), LactoBif 100 (1 cap, intermittent)	Same	Endocrine and GI primer. Zinc supplement removed.
04:00 Wake Vector	Vector V3 RECOVERY	Vector V3 RECOVERY (when consumed)	Rapid amino acid delivery and green flush.
05:00 First Meal	Roasted kasha breakfast, Earl Grey tea; Kyolic 104 (2 caps), NAC 900 mg, NMN 175 mg, Digestive Enzymes Ultra (1 cap)	Main meal; Digestive Enzymes Ultra (1 cap). Grid pills skipped on rest days.	Solid fuel carbohydrate baseline.
Pre-Gym T-30 / 05:30-06:00	Citicoline 250 mg; Vector V1 IGNITION at 06:00	Inactive	CNS stimulation and pump synthesis; spaced about 2 hours post-tea.
09:30 Intra- Workout	Vector V2 REFUEL; 2-3 Medjool dates chewed separately	Inactive	Glycogen replenishment and electrolyte preservation.
11:00 Post- Workout	Whole R-ALA 300 mg with water, then Vector V4 RELOAD	Inactive	R-ALA taken whole; never emptied into the shake.
12:00 Lunch / Meal 2	Training Lunch Pill Grid; Digestive Enzymes Ultra (1 cap)	Main meal; Digestive Enzymes Ultra (1 cap)	Micronutrient repletion. Entire grid skipped on rest days.
15:00-16:30 Afternoon	Training Afternoon Pill Grid	Vector V5 TRANSPORT (16:00-16:30, if consumed)	Second creatine pulse on rest days via V5.
18:15 Night Stage 1	Pure Magnesium Glycinate 240 mg, Tart Cherry 10X (1 cap), Milk Thistle (1 cap, conditional)	Skipped	Neuromuscular recovery and sleep architecture.
18:30 Night Stage 2	Vector V10 SHUTDOWN (hibiscus base; optional half Medi-C; rare Hey Relax)	Vector V10 SHUTDOWN (same options)	Collagen synthesis, night hydration, parasympathetic transition.

Analysis 4A - circadian anchors. Consistent wake, light, caffeine and training times entrain sleep better than minute-perfect nutrient timing. The chronometer's value is repeatability: the same anchors let BIA, performance and digestion logs be compared week to week. The evening PUMP branch is strictly inactive and must never be combined with a morning stimulant session.

Chapter 5 - Solid fuel and nutrition records

Known totals are partial label calculations, not a measured whole diet.

Whole-day caloric, macro- and micronutrient verification remains deferred in V18700. Historical baselines designating 2,780 kcal, 252 g protein and 69 g fat are preserved as legacy planning figures rather than rigid physiological targets. Every modern total in this Encyclopedia is printed with its limit beside it.

Meal / module	Composition (recorded)	Published historical macros	Clinical relevance
Breakfast / Primer	About 80 g dry roasted kasha, Earl Grey tea	Deferred (not verified)	Carbohydrate baseline; tea timed about 2 hours before V1 to limit tannin and caffeine stacking confusion.
Training Lunch	Main meal + Lunch Pill Grid + Digestive Enzymes Ultra	Deferred	Largest solid-fuel window on training days.
MS-01 Mega Shake (Preserved Legacy)	500 mL Protein 3% ultrafiltered milk, 2 scoops Hexapro, 2 scoops PB&Me, hydrated chia, bovine collagen, bone broth protein, cocoa, berries	1,115 kcal / 129 g P / 73 g C / 36 g F (label check 1,132 kcal)	High-volume meal replacement. Bone-broth protein content remains an unverified label claim.
SB-01 Salad Bowl (Preserved Legacy)	Tuna, feta, jumbo beans, avocado oil in 32-40 oz bowl	810 kcal / 50 g P / 55 g C / 36 g F (check 744 kcal)	High micronutrient density; fat moderated to digestive tolerance post-cholecystectomy.
FUSION_2A Dark Matter Casein (Legacy)	2 scoops Hexapro Chocolate, 175 g Isey 0% Skyr, All-Bran Buds, wild blueberries	480 kcal / 50 g P / 45 g C / 8 g F (check 452 kcal)	Nighttime slow-digesting blend. No verified mandatory 19:00 curfew.

Analysis 5A - GI physiology without a gallbladder. Continuous bile trickle favours smaller, more frequent fat loads over single large boluses. Supplemental magnesium (341 mg baseline, up to 446-646 mg with Medi-C or Hey Relax) exceeds the 350 mg supplemental upper level and, combined with high-dose vitamin C, creates an osmotic chain that compounds bile-related looseness. Stool changes are therefore logged as system data, not personal failure.



Chapter 6 - Training mechanics and movement classes

Movement classes are defined by direction of force, joint angles and loading demands - distinct from intake vectors V1-V10.

Supported rows, presses and machine-based work dominate where hardware locks require spinal protection. Free axial loading is progressed conservatively. Tempo, setup repeatability and pain response are recorded for every primary movement. The 2026 ACSM update supports individualized resistance training prescription; it does not endorse any single Project 100 template for every reader.

Movement class	Examples in system	Loading logic
Supported horizontal pull	Chest-supported row	High volume tolerance; minimal spinal shear.
Supported press	Machine / dumbbell press with back support	Shoulder tolerance governs range and grip.
Hinge (modified)	Reduced-range hinge as tolerated	Symptom-guided; stopped by neurological signs.
Squat pattern (supported)	Leg press / supported squat	Knee and back response logged per session.
Isolation / corrective	Elbow- and shoulder-friendly isolation	Control and blood flow without axial cost.

Analysis 6A - load management. Hypertrophy requires progressive mechanical tension with adequate recovery. Volume landmarks are individual: the correct dose is the largest dose that leaves performance stable or improving across the week with hardware locks quiet. Soreness alone is not a progress metric.

Chapter 7 - The failure matrix and correction method

Diagnose before escalating: measurement, sleep, digestion and load are checked before any dose changes.

Symptom code	First checks	Correction before escalation
Stalled SMM estimate	Same-device conditions? Hydration, glycogen, recent training?	Repeat standardized BIA over 2-3 weeks; review performance trend.
Poor sleep	Caffeine total and timing, TeaCrine half-life (16-20 h), awakenings	Pull caffeine earlier; do not add sedative stacks reflexively.
Loose stools	Magnesium total, vitamin C dose, fat bolus size, bile pattern	Split fat, review osmotic stack with clinician.
Joint / nerve pain flare	New exercise, range or load jump?	Return to last tolerated setup; assess if progressive.
Afternoon energy crash	Sleep debt, post-workout carbohydrate (V4 only 19 g), ALA timing	Address sleep and fuel before stimulants.

Suggested reductions are not adopted changes until the user confirms them. A removed item cannot reappear because an old page includes it.

Chapter 8 - Supplements, pills and micronutrient accounting

23 pill records audited. Every dose below reproduces the V18700 operational record; the analysis column states the evidence limit.

Block	Recorded items and doses	Status	Evidence limit / watch
Wake Pack 03:30	TESTOFX 3 caps (incl. BioPerine 9.5 mg piperine); Boron 1 x 3 mg; LactoBif 100 1 cap	Daily; probiotic intermittent	Fasting testosterone release not clinically established for this blend.
Meal 1 05:00	Kyolic 104 2 caps; NAC 900 mg; NMN 175 mg; Digestive Enzymes Ultra 1 cap	Training and rest (enzymes)	NAC and NMN human outcome data limited; enzymes support tolerance, do not replace bile.
Pre-Gym	Citicoline 250 mg whole capsule	Training only	Contributes to cholinergic stack (see Part IV).
Lunch Grid (training only)	Multivitamin (incl. choline 57 mg, biotin share), Curcumin 500 + BioPerine 5 mg, Alpha-GPC 200 mg, DMAE, Huperzine A 200 mcg	Skipped on rest days	Piperine total reaches 14.5 mg/day on training days. Cholinergic load is the decisive watch.
Afternoon Grid (training only)	Co-factors per V18700 grid	Skipped on rest days	Grid skipped on rest days is a deliberate de-load of micronutrient stacking.
Night Stage 1 18:15	Pure Magnesium Glycinate 240 mg; Tart Cherry 10X 1 cap; Milk Thistle 1 cap conditional	Training only	Supplemental magnesium must be summed across all sources (Part V).
Removed / paused	Zinc Supreme REMOVED (30 mg zinc gone); separate copper PAUSED	Do not reintroduce from old pages	Removal corrected zinc excess risk; copper paused with it.
Optional	Half-dose Medi-C (adds 1,000 mg vitamin C, 105 mg Mg); Hey Relax full scoop, rare (adds 200 mg Mg, 1,000 mg inositol)	Optional / rare only	Never promoted to daily requirements in this edition.

Analysis 8A - why the grid is skipped on rest days. Skipping the lunch and afternoon grids on rest days halves weekly exposure to piperine, cholinergic precursors and fat-soluble stacking. That rhythm is protective. Eroding it for marginal micronutrient completeness would trade a real risk reduction for an unproven benefit.

Chapter 8 (continued) - Dose accounting with limits beside them

Compound	Training day	Rest day	Limit / reference	Assessment
Biotin (total)	425 mcg	425 mcg	RDA 30 mcg; no UL	Assay interference risk, not toxicity (Part VI).
Supplemental magnesium	341 mg baseline; 446 mg with half Medi-C; 541-646 mg with Hey Relax	Lower (night grid skipped; V10 option still counts)	UL 350 mg supplemental	Baseline near UL; optionals exceed it. Osmotic effect expected.
Caffeine (IGNITER)	About 373 mg net, plus Earl Grey 40-70 mg	Tea only	Health Canada 400 mg/day guidance	Training mornings regularly cross guidance when tea is summed.
Piperine	14.5 mg (9.5 + 5)	9.5 mg (TESTOFX only)	No established UL	CYP3A4 / P-glycoprotein inhibition is dose-relevant (Part IV).
Creatine HCl	1.5 g (V4 750 mg + V10 750 mg)	1.5 g (V5 750 mg + V10 750 mg)	No UL; saturation dose	Expands creatinine pool; eGFR interpretation needs Cystatin C.
Boron	3 mg (1 capsule)	3 mg	UL 20 mg	Locked at one capsule; wide margin.
R-ALA	300 mg whole capsule pre-V4	None	No UL	IAS flag in susceptible HLA carriers (Part IV).

Dose reconciliation notes: the Canadian Carnivor panel contains zero added creatine - older assumptions crediting 2.5 g creatine to V3 are disproven. V4 is deliberately half the BioSteel label serving (one 36 g scoop: 12 g protein, 19 g carbohydrate, 125 kcal). RapiDrem Electrolyte Powder replaces BioSteel Hydration in V2 and V5 (1,000 mg potassium, 100 mg magnesium per scoop).



Chapter 9 - Planning over months and years

Quarterly review beats weekly overreaction.

Quarterly body-composition review is the project cadence. The timing and scope of blood testing follow clinical need, not the training calendar. A few comparable weigh-ins describe a week; an isolated SMM or body-fat reading should not trigger a new diet or supplement change.

Planning horizons: weekly (performance, sleep, digestion log), quarterly (standardized BIA trend, waist, photos under consistent light), annually (full laboratory panel timed with the clinician, medication and supplement reconciliation). Brand changes and label versions are logged with dates, because a product name is not a formula guarantee.

Analysis 9A. Adaptation is slow and noisy. The planning system exists to prevent the most common error in self-directed programmes: changing three variables after one noisy measurement and learning nothing. One change, one review window, one decision.

Chapter 10 - Connective tissue and injury considerations

Collagen, vitamin C, glycine and load progression support tolerance; no supplement proves nerve healing.

Daily collagen exposure is substantial and deliberate: 10 g marine collagen in V3, 10 g in V5 on rest days, plus bovine collagen in legacy meal modules and glycine 3-5 g plus glutamine 5 g in V10. Vitamin C (including optional Medi-C) is the required co-factor for collagen cross-linking. The system pairs substrate with progressive, symptom-guided loading - the stimulus that actually directs tissue adaptation.

Elbow, shoulder and spinal history require exercise substitution rather than bravado. Supported positions, controlled tempo and range limited by symptoms are the connective-tissue strategy. Tart cherry and milk thistle in the night stage are recorded as used; their contribution cannot be isolated from sleep and load management.

Chapter 11 - Hormonal and metabolic interpretation

An afternoon testosterone value is a trough, not a diagnosis.

The March 2026 testosterone of 7.4 nmol/L drawn at 13:28 is a non-diagnostic afternoon trough. Endocrine Society and American Urological Association guidance requires two fasting morning draws (07:00-09:30) to diagnose deficiency, because diurnal decline can halve afternoon values in younger men and blunts with age but does not disappear.

Metabolic interpretation in this system is conservative: body fat below 15 percent is an aspiration, not a prerequisite for insulin sensitivity. Meal enzymes, fibre and meal sequencing are logged as tolerance tools. Any hormonal intervention is a clinical decision outside this Encyclopedia's authority.

Analysis 11A. NMN (175 mg), boron (3 mg) and TESTOFX are recorded as used. None carries human evidence sufficient to promise testosterone restoration. The Encyclopedia records; it does not prescribe endocrine therapy.

Chapter 12 - The singularity philosophy

Durability outranks comparison.

The singularity in Project 100 is not a physique borrowed from a stage division. It is the point where measurement, training, digestion, sleep and laboratory honesty operate as one system, sustained for years. The author's own comparable measurements, function and health are the scoreboard.

"Train Like Two. Eat for Two" survives as the motto of effort. The mature reading is narrower and stronger: train with intent, eat with record, rest without guilt, and let symptoms - not ambition - set the ceiling on any given day. A larger stack, longer session or bigger meal does not prove better adaptation.

This philosophy chapter closes the operational book. The parts that follow - vectors, cascades, margins and laboratory playbook - are the reference engine that keeps the philosophy honest.

Part III - Vector deep dives: liquid payload architecture

Intake vectors are formulated liquid payloads with exact gram masses, fluid bases and delivery routes. V1, V2, V3, V4, V5 and V10 are active recipes. PUMP is preserved for reference and strictly inactive.

Vector	Timing and status	Base	Complete formulation (recorded)	Target mechanism and reconciliation
V3 RECOVERY	04:00; training and rest	Cold filtered water; shaker volume unspecified	MuscleMeds Carnivor Beef Protein Isolate (33.9 g Canadian scoop); PROMISE Pure Marine Collagen Peptan 10 g; Rootalive Organic Gelatinized Black Maca 5 g; ALLMAX Liquid L-Carnitine 1500 (15 mL / 1,500 mg); MegaFood C Defense Gummy (1, chewed); NAKA Platinum Vital Greens (20 mL separate flush)	23 g beef protein, 10 g marine collagen, 1.5 g free carnitine, whole-food ascorbate. Canadian Carnivor panel: zero added creatine. Greens taken as separate 20 mL flush.
V1 IGNITION	06:00 pre-gym; morning training only	500-600 mL cold filtered water	ALLMAX Impact Igniter Xtreme 2.0 (2 scoops / 28 g); ALLMAX MuscleEAA Xtreme (1 scoop / 17.73 g); PeakO2 2,000 mg; Betaine Anhydrous 2,500 mg; Citrulline Malate 2:1 (4-6 g); Pure Taurine Powder (2-3 g). Citicoline 250 mg capsule taken separately.	About 373 mg total caffeine, 10.67-12 g active citrulline base, 3.2 g beta-alanine, 2 g PeakO2 mushrooms, 2.5 g betaine. Spaced about 2 hours post-breakfast tea.
V2 REFUEL	09:30 intra-workout; training only	600-750 mL cold filtered water	Perfect Sports BCAA HyperClear (1 scoop / 7.5 g BCAAs 2:1:1); RapiDrem Electrolyte Powder (1 scoop, about 6.5 g); Kirkland Himalayan Pink Salt 1/16 tsp; 2-3 Medjool dates chewed solid, separately	3,750 mg leucine-led BCAAs, 1,000 mg potassium, 100 mg magnesium, about 36-54 g fast carbohydrate from dates.

Part III (continued) - V4, V5, V10 and the inactive PUMP

Vector	Timing and status	Base	Complete formulation (recorded)	Target mechanism and reconciliation
V4 RELOAD	11:00 post-workout; training only. Preceded by whole R-ALA 300 mg with water	350-450 mL room-temperature water	BioSteel Recovery Protein Plus (1 scoop / 36 g half-serving); Rootalive Beetroot Powder 7 g raw; Proline Pure Creatine HCl 750 mg active; Kirkland Himalayan Salt 1/16 tsp	Scaled half-serving: 12 g protein, 19 g carbohydrate, 125 kcal. Dietary nitrates from beetroot combine with creatine HCl. R-ALA swallowed whole; emptying into the shake risks mucosal irritation and degradation.
V5 TRANSPORT	16:00-16:30; rest branch only	500 mL cold filtered water	Proline Pure Creatine HCl 750 mg; PROMISE Pure Marine Collagen 10 g; ALLMAX Liquid L-Carnitine 1500 (15 mL / 1,500 mg); RapiDrem Electrolyte Powder (1 scoop); Kirkland Himalayan Salt 1/16 tsp	Rest-day bridge: second daily creatine pulse (750 mg), 10 g connective-tissue peptides, carnitine and electrolytes.
V10 SHUTDOWN	18:30; training and rest	800 mL Traditional Medicinals Hibiscus Tisane, comfortable temperature	SD Pharmaceuticals Creatine HCl 750 mg; North Coast Naturals Fermented L-Glutamine 5 g; AOR Pure Glycine Powder 3-5 g; optional half-scoop Medi-C Plus (2.5 g); rare Niyama Hey Relax full scoop	Nocturnal recovery: second creatine pulse, 5 g glutamine, 3-5 g glycine. Optional Medi-C adds 1,000 mg vitamin C and 105 mg magnesium; Hey Relax adds 200 mg magnesium and 1,000 mg inositol. The old 60-65 degree Celsius heating rule is retired.
PUMP EVENING	Inactive. Never used with V1	500-600 mL cold water	ALLMAX Impact Pump Xtreme (2 scoops / 31 g); MuscleEAA Xtreme (1 scoop); PeakO2 2,000 mg; Betaine 2,500 mg; Citrulline Malate 2:1 (4-6 g); Taurine 2-3 g; Citicoline 250 mg	Preserved strictly for structural reference. Non-stimulant evening alternative that must never be co-administered with a morning stimulant session.

Formulation adjustments locked in V18700: RapiDrem replaces BioSteel Hydration in V2 and V5. V4 stays at the reduced half-serving to limit refined maltodextrin. Carnivor contributes no creatine. Temperature mandates for V10 are retired - comfortable temperature only.

Part IV - The five interaction cascades: master matrix

The operational record is sound. Risk concentrates in five interactions, not in any single ingredient. Read this matrix before changing any dose.

#	Cascade	Components and summed dose	Mechanism	Practical consequence
1	Piperine drug-clearance inhibition	TESTOFX BioPerine 9.5 mg + Curcumin BioPerine 5 mg = 14.5 mg/day (training days)	Inhibits intestinal P-glycoprotein and hepatic CYP3A4 plus phase-II glucuronidation	Raises systemic exposure to co-administered drugs and to the stack itself. Disclose to prescribers.
2	Cholinergic pressor stack	Citicoline 250 mg + Alpha-GPC 200 mg + choline salts + DMAE + multivitamin choline 57 mg, against Huperzine A 200 mcg	Simultaneous precursor loading plus acetylcholinesterase inhibition	Excess cholinergic tone: bradycardia, fasciculations, salivation, GI cramps that can mimic overtraining.
3	ALA to insulin autoimmune syndrome	R-ALA 300 mg + glycogen-depleted training + only 19 g carbohydrate in V4	Health Canada flags alpha-lipoic acid for IAS (Hirata disease) in HLA-DRB1*0406 / *0403 carriers	Hypoglycaemia symptoms post-training need glucose awareness, not stimulant escalation.
4	Osmotic GI stack without a gallbladder	Magnesium 341-646 mg + high-dose vitamin C + continuous bile trickle	Osmotic water draw compounds bile-related stool looseness	Loose stools are predictable system output; log and moderate fat bolus before blaming single foods.
5	Stimulant stack at the ceiling	IGNITER about 373 mg caffeine + TeaCrine (theacrine half-life 16-20 h) + Dynamine + theobromine + Earl Grey 40-70 mg	Overlapping methylxanthine exposure across the day	Exposure regularly crosses the 400 mg/day guidance; sleep is the limiting organ.

Rule: never treat a cascade symptom by adding its opposite drug class from a supplement shelf. Bradycardia, recurrent hypoglycaemia symptoms, jaundice or progressive weakness are clinical presentations.

Part V - Toxicological margins: master table

A Project 100 number is never printed without its limit. This table is the single margin reference for the whole system.

Substance	System exposure	Reference limit	Margin verdict
Caffeine (training morning, incl. tea)	About 413-443 mg	Health Canada 400 mg/day	At or above guidance. Sleep governs.
Supplemental magnesium	341 mg baseline; up to 646 mg with optionals	UL 350 mg/day supplemental	Baseline at UL; optionals exceed. Expect osmotic effect.
Vitamin C (with half Medi-C)	Baseline plus 1,000 mg	UL 2,000 mg/day	Within UL; GI effect is the practical limit.
Boron	3 mg	UL 20 mg/day	Wide margin. Locked at one capsule.
Biotin	425 mcg	No UL; RDA 30 mcg	Toxicity margin wide; assay margin zero - disclose.
Piperine	14.5 mg training / 9.5 mg rest	No UL established	Interaction margin governs, not toxicity.
Creatine HCl	1.5 g/day both branches	No UL	Renal interpretation margin: use Cystatin C eGFR.
R-ALA	300 mg training days	No UL	IAS susceptibility governs, not dose toxicity.

Toxicology in this system is interaction toxicology. Single-ingredient margins look comfortable; summed exposures - caffeine, magnesium, piperine, cholinergic load - are where the Encyclopedia directs attention.

Part VI - Diagnostic laboratory integrity playbook

The three measurement confounders

Confounder	Mechanism	Distorted results	Corrective action
Biotin 425 mcg/day	Streptavidin-biotin immunoassay interference	Falsely high free T4/T3, testosterone, cortisol; falsely low TSH, troponin, PTH	Tell the laboratory. Follow assay-specific hold instructions. No universal 72-hour rule.
Creatine HCl 1.5 g/day	Expanded creatinine pool	Depressed creatinine-based eGFR without renal injury	Request Cystatin C eGFR for renal interpretation.
Afternoon testosterone	Diurnal decline	7.4 nmol/L at 13:28 reads as deficiency falsely	Two fasting morning draws, 07:00-09:30, on separate days.

Playbook rules

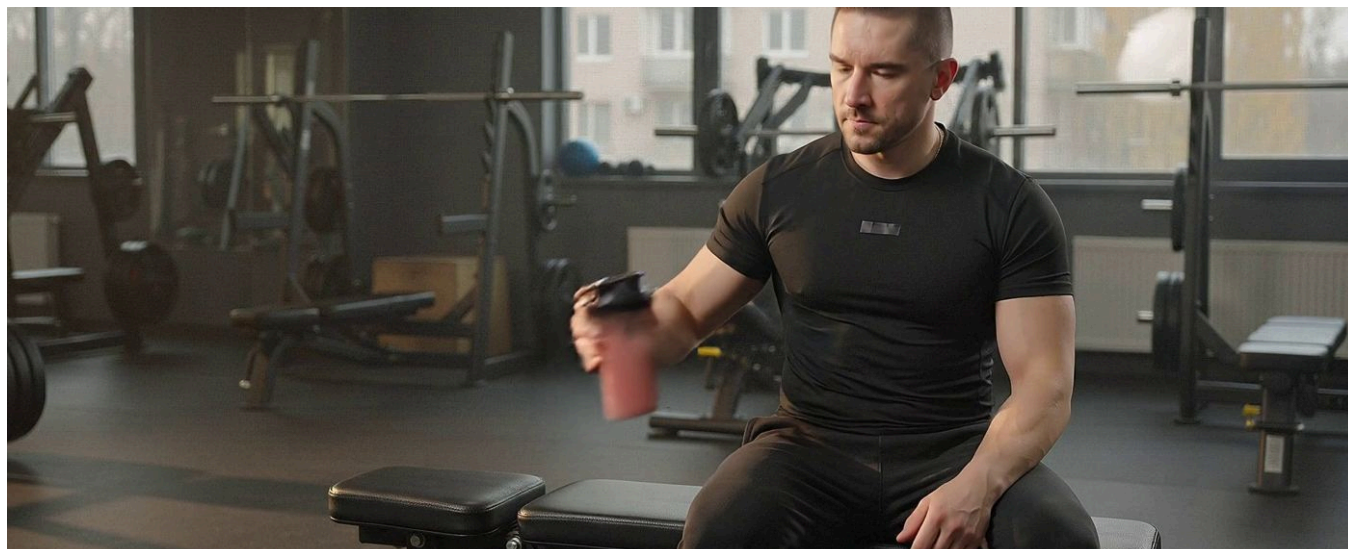
- March 2026 laboratories predate the May supplement stack. Never cite them for or against the current stack.
- Record creatine, biotin and caffeine on the laboratory requisition note or patient portal message.
- Creatine kinase (CK) after hard training is expected to rise; time draws away from peak sessions when CK interpretation matters and tell the clinician the training date.
- Any troponin concern bypasses supplement reasoning entirely: emergency assessment first.

Monitoring plan: quarterly standardized BIA and waist; sleep, digestion and performance logs weekly; laboratory panel timed to clinical need with the confounders above disclosed. That cadence converts this Encyclopedia from a record into a surveillance system.

Annex I - Tactical tables and implementation grid

Time	Training day - execute	Rest day - execute
03:30	TESTOFX 3 caps, Boron 3 mg, LactoBif intermittent	Same
04:00	V3 RECOVERY	V3 RECOVERY (when consumed)
05:00	Kasha breakfast, tea, Kyolic, NAC, NMN, Enzymes	Main meal, Enzymes. Grid skipped.
06:00	Citicoline + V1 IGNITION, train	No V1. Sleep in / walk.
09:30	V2 REFUEL + dates during session	-
11:00	R-ALA whole, then V4 RELOAD	-
12:00	Lunch + Lunch Grid + Enzymes	Main meal + Enzymes
15:00-16:30	Afternoon Grid	V5 TRANSPORT (if consumed)
18:15	Mg Glycinate, Tart Cherry, Milk Thistle conditional	Skipped
18:30	V10 SHUTDOWN (options as recorded)	V10 SHUTDOWN (options as recorded)

Implementation rules: PUMP never combined with V1. Zinc Supreme stays removed. Copper stays paused. Boron stays at one capsule. Medi-C stays at half-dose optional. Hey Relax stays rare. Any new product enters the log with brand, market label, serving definition and start date before it enters the body.



Annex II - Operator checklists and red-flag checklist

Daily operator checklist

- Sleep hours and awakenings recorded. Caffeine total estimated including tea.
- Branch selected correctly: training grid only on training days.
- R-ALA swallowed whole with water before V4 (training days).
- Stool, pain, weakness and performance noted in one line each.
- Optional items (Medi-C, Hey Relax) marked as taken or skipped - never assumed.

Weekly review (15 minutes)

- Performance trend stable or improving? Hardware locks quiet?
- One variable changed at most. Next review date set.

Red-flag checklist - for clinician discussion. Stop and seek assessment for:

- Progressive hand or limb weakness, new numbness, bowel or bladder change.
- Jaundice, dark urine with pale stool, fever with abdominal pain.
- Recurrent watery stools with weight loss, oily stools, significant abdominal pain.
- Palpitations, fainting, unusual slow pulse, muscle twitching with GI cramps (cholinergic watch).
- Recurrent sweating, tremor or confusion after training (hypoglycaemia watch on R-ALA days).
- Sleep disruption that does not resolve when caffeine is pulled earlier.

Bring this Encyclopedia, the current product labels and the laboratory playbook (Part VI) to the appointment. The clinician needs the stack as recorded, not as remembered.

Sources, evidence limits and version history

Source register

- Platinum Omnibus V18700, published 30 September 2026 (76 pages) - source of truth for all doses, recipes and statuses reproduced here.
- Project 100 Protocol Analysis / Comprehensive Protocol Audit and Clinical Pharmacology Analysis (33 pages, September 2026) - source of the mechanism, cascade, margin and laboratory layer integrated in V2.0.
- Mega Encyclopedia V187.01, Simplified Structure V187.01 and Current Correction Handoff V187.01 (DOCX) - structural inputs reconciled against V18700.
- Author records: age 46, body weight 90.76 kg (current profile); February baseline age 45, 197.1 lb, SMM 88.2 lb, body fat 22.9 percent. March 2026 laboratories (pre-stack).

Evidence limits

Evidence tags used across the V2.0 set: ESTABLISHED (clinical consensus), SUPPORTED (human trial evidence), PLAUSIBLE (mechanistic rationale), UNPROVEN (marketing extrapolation). Product claims - including bone-broth protein content in legacy meal modules - remain unverified label claims where flagged. BIA, testosterone timing and biotin interference statements reflect the assay and physiology literature summarised in the Protocol Analysis. This Encyclopedia documents one person's system; it does not establish that the system is safe or effective for any other person.

What was corrected and locked in V18700 / V2.0

- Zinc Supreme removed (30 mg zinc). Separate copper paused. Boron locked at 1 x 3 mg.
- V4 fixed at half the BioSteel serving. Carnivor confirmed at zero added creatine (Canadian panel).
- RapiDrem replaces BioSteel Hydration in V2 and V5. V10 heating mandate retired.
- PUMP branch inactive. Training pill grids skipped on rest days.
- Retired myths removed: hot-water bile flush, calcium-neutralises-zinc, mandatory mineral firewall.

Version history and closing

Version history

Version	Date	Change
V18700	30 Sept 2026	Platinum Omnibus published. Operational record locked.
V187.01 drafts	Early Oct 2026	Mega, Simplified and Handoff reconciliations.
V2.0 derivatives	3 Oct 2026	Full Detailed, Complete Clinical Analysis, Guidebook, Simplified Structure, Slider, Website documents.
V2.0 Encyclopedia (this volume)	3 Oct 2026	Full Detailed + Complete Clinical Analysis merged. The master from which all derivatives are generated.

End of the Encyclopedia. The system this book records is deliberately unglamorous: measure honestly, train repeatably, eat with a record, sleep, and let symptoms set the ceiling. PROJECT 100 - 100 lb of skeletal muscle mass, body fat below 15 percent - remains the horizon. Durability is the method.

PROJECT 100 ENCYCLOPEDIA V2.0 - 2026-10-03 - Bogdan Dospi - project100.fit - Personal document. Not medical advice.

PROJECT 100 ENCYCLOPEDIA V2.0 - 2026-10-03 - Bogdan Dospi